INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Leuprorelin 22.5 mg: hormone injection every 12 weeks

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
84 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00479
Version 3

One injection every 12 weeks

Treatment stages

01 **Long-acting hormone treatment**
One injection every 12 weeks

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Advanced disease, or selected high-risk disease with radiotherapy

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call your team promptly for new or worsening symptoms, severe low mood or problems taking treatment. New leg weakness, loss of bladder or bowel control, severe chest pain, severe breathing difficulty, collapse or a seizure needs emergency assessment: call 112/999.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

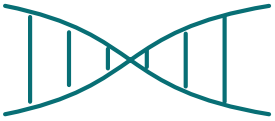
How this hormone treatment works

This is androgen deprivation therapy (ADT): it lowers testosterone to help control prostate cancer. With radiotherapy, the aim may be cure; in advanced disease, it is part of ongoing cancer control.

GNRH AGONIST

Leuprorelin 22.5 mg

Eligard



Reduces testosterone through sustained pituitary stimulation

Leuprorelin is a GnRH agonist. Testosterone can rise briefly at the start before continued stimulation switches down the pituitary signal. The depot then maintains suppression between injections. The team assesses flare protection and confirms that testosterone is adequately suppressed.

Brief initial stimulation → Pituitary signal switches down → Testosterone falls

What to watch for

Confirm the exact next injection date. With radiotherapy, ask when hormone treatment starts and ends and how the radiotherapy dates fit. A low PSA does not mean you should stop injections. Tell the team about worsening pain, trouble passing urine or severe headache with vision changes.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

What the evidence can tell you

Trials support the discussion but do not predict an individual outcome.

For selected high-risk localised or locally advanced prostate cancer, ADT is combined with radiotherapy to improve cancer control. In advanced disease it forms the hormone-treatment foundation. Duration and additional medicines depend on the setting; this information does not establish one universal course or a survival ranking among depot products.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment course

Leuprorelin 22.5 mg: hormone injection every 12 weeks

84-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

ONE INJECTION EVERY 12 WEEKS

Long-acting hormone treatment

Duration set by your treatment plan

Leuprorelin 22.5 mg is injected under the skin every 12 weeks (84 days) in this NCCP regimen. The prescribed duration depends on whether it supports radiotherapy or treats advanced disease.

Androgen deprivation therapy (ADT) lowers testosterone, a hormone that can drive prostate cancer. This medicine is your ADT. Additional cancer tablets, chemotherapy or radiotherapy are separate decisions based on your cancer and overall health. Keep treatment going until your team changes the plan; do not stop because prostate-specific antigen (PSA) has fallen.

BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Safety and response tests have different purposes. Use the individual prescription and appointment dates.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00479 v3: official regimen · Eligard: product information

Tests in your plan

Blood count, kidney and liver tests

WHEN

Repeat blood count, kidney and liver tests, glucose and bone blood profile when clinically indicated. PSA and testosterone assess response. A suspected preparation or administration problem needs review and a testosterone check.

Checks blood cells and organ health.

Blood glucose and bone blood profile

WHEN

Repeat blood count, kidney and liver tests, glucose and bone blood profile when clinically indicated. PSA and testosterone assess response. A suspected preparation or administration problem needs review and a testosterone check.

Checks metabolic effects and blood minerals.

PSA and testosterone

If part of your plan

WHEN

Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.

Checks cancer activity and hormone suppression.

ECG and bone-density assessment

If part of your plan

WHEN

ECG (heart tracing) and bone-density scans are arranged according to heart-rhythm risk, fracture risk and clinical need. Your team confirms which tests you need.

Checks heart rhythm or fracture risk when indicated.

THE WHOLE TREATMENT JOURNEY

Leuprorelin 22.5 mg: hormone injection every 12 weeks: treatment and checks

Hormone-dependent advanced prostate cancer, or high-risk localised/locally advanced hormone-dependent disease in combination with radiotherapy.

Leuprorelin 22.5 mg is injected under the skin every 12 weeks (84 days) in this NCCP regimen. The prescribed duration depends on whether it supports radiotherapy or treats advanced disease.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	Between appointments	Next review	Longer-term care
Blood and organ safety NCCP · SMPC	Full blood count, kidney and liver tests, blood glucose and a bone blood profile. Review heart disease, blood pressure, diabetes, mood, falls and fracture risk, sexual health, medicines and the treatment goal.	Confirm baseline results and the starting plan.	Repeat blood count, kidney and liver tests, glucose and bone blood profile when clinically indicated. PSA and testosterone assess response. A suspected preparation or administration problem needs review and a testosterone check.	Confirm results and symptoms before the next prescription or injection.	Review bone, metabolic and heart health.
Treatment and appointments NCCP · SMPC	Agree treatment aim, formulation, duration and medicines list.	A trained healthcare professional prepares and gives this specific formulation under the skin. Correct mixing and administration matter for hormone suppression. Different leuprorelin products and strengths have different instructions; they must not be substituted by matching the number of milligrams alone.	Keep an accurate medicine / injection record.	Confirm the exact next injection date. With radiotherapy, ask when hormone treatment starts and ends and how the radiotherapy dates fit. A low PSA does not mean you should stop injections. Tell the team about worsening pain, trouble passing urine or severe headache with vision changes.	Continue until the team changes the plan.

Care / check	Before starting	First treatment	Between appointments	Next review	Longer-term care
Cancer response NCCP · SMPC	Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.	Document the baseline PSA and clinical picture.	Ask when the next PSA and any testosterone test are due.	Review symptoms and results together.	Scans follow the individual plan; there is no universal interval.

What each check is for

1



Treatment and review

One injection every 12 weeks

This is androgen deprivation therapy (ADT): it lowers testosterone to help control prostate cancer. With radiotherapy, the aim may be cure; in advanced disease, it is part of ongoing cancer control.

Ask / **act:** Repeat blood count, kidney and liver tests, glucose and bone blood profile when clinically indicated. PSA and testosterone assess response. A suspected preparation or administration problem needs review and a testosterone check.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before starting

Full blood count, kidney and liver tests, blood glucose and a bone blood profile. Review heart disease, blood pressure, diabetes, mood, falls and fracture risk, sexual health, medicines and the treatment goal.

STEP 2

Taking or receiving treatment

A trained healthcare professional prepares and gives this specific formulation under the skin. Correct mixing and administration matter for hormone suppression. Different leuprorelin products and strengths have different instructions; they must not be substituted by matching the number of milligrams alone.

STEP 3

Tests and reviews

Repeat blood count, kidney and liver tests, glucose and bone blood profile when clinically indicated. PSA and testosterone assess response. A suspected preparation or administration problem needs review and a testosterone check.

STEP 5

Leaving with a clear plan

Confirm the exact next dose or appointment, duration and verified daytime and out-of-hours contacts.

Your exact treatment schedule

Tests and preparation

PSA and testosterone

Prostate-specific antigen (PSA) helps assess cancer activity. Testosterone checks the effect of hormone suppression, especially if response is uncertain. Your team interprets these with symptoms and scans.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Your 12-week injection interval

One injection or implant at the start. The medicine continues working between visits.

Treatment or check	Timing in this view
Injection / implant	**Week 1** Day 1
Continuous hormone suppression	**Week 1 → Week 2 → Week 3 → Week 4 → Week 5 → Week 6 → Week 7 → Week 8 → Week 9 → Week 10 → Week 11 → Week 12** Depot works throughout all 12 weeks

Prescribed schedule: Leuprorelin 22.5 mg is injected under the skin every 12 weeks (84 days) in this NCCP regimen. The prescribed duration depends on whether it supports radiotherapy or treats advanced disease.

How this fits: Androgen deprivation therapy (ADT) lowers testosterone, a hormone that can drive prostate cancer. This medicine is your ADT. Additional cancer tablets, chemotherapy or radiotherapy are separate decisions based on your cancer and overall health. Keep treatment going until your team changes the plan; do not stop because prostate-specific antigen (PSA) has fallen.

Early safety: Confirm the exact next injection date. With radiotherapy, ask when hormone treatment starts and ends and how the radiotherapy dates fit. A low PSA does not mean you should stop injections. Tell the team about worsening pain, trouble passing urine or severe headache with vision changes.

> The next dose is due 84 days after the previous dose unless the team changes the plan. Twelve weeks is not always the same date as three calendar months. Use the written appointment date.

Between-treatment notes

Day 1: Treatment visit

A trained healthcare professional prepares and gives this specific formulation under the skin. Correct mixing and administration matter for hormone suppression. Different leuprorelin products and strengths have different instructions; they must not be substituted by matching the number of milligrams alone. Leuprorelin 22.5 mg is injected under the skin every 12 weeks (84 days) in this NCCP regimen. The prescribed duration depends on whether it supports radiotherapy or treats advanced disease.

Day 2–84: Between appointments

The depot continues working between injections. Symptoms can occur on any day. Keep your next booked appointment; contact the team promptly if it is missed.

Support around the medicines

1 Sickness support

This treatment has minimal sickness risk. Routine preventive anti-sickness medicines are not usually required; agree a plan if symptoms develop.

2 **Bone and muscle health**

Discuss fracture risk, bone-density testing when indicated, calcium intake, vitamin D and suitable resistance and aerobic exercise. Supplements and bone medicines are individual decisions.

3 **Sexual health, mood and hot flushes**

Tell the team which effects matter most. Support is available for sexual function, sleep, mood and vasomotor symptoms.

4 **Medicine review**

Bring all prescriptions, over-the-counter medicines and supplements. Ask pharmacy before starting or stopping anything.

Why hormone treatment is used

Treatment goals and choices

Additional treatment

In metastatic hormone-sensitive cancer, many suitable patients benefit from an additional androgen-pathway medicine, and sometimes chemotherapy. Radiotherapy can have a different role in localised disease, selected metastatic disease or symptom relief. Your specialist explains the options for your setting.

Deciding together

Discuss expected benefit, uncertainty, side effects, alternatives including surgical castration where appropriate, and the option of declining treatment. No one duration fits every setting.

Questions worth asking

Do I need another cancer medicine or radiotherapy alongside this?

Ask how this hormone treatment fits the plan for your disease. Additional hormone-pathway medicines, chemotherapy and radiotherapy have different roles; they are not automatically needed in every setting.

How can we protect bone, heart, sexual and mental health?

Ask for a plan covering exercise, bone and fracture risk, blood pressure and glucose, sexual symptoms, mood and sleep. Tell the team about changes early so support can be offered.

Who can I contact urgently?

Use the verified daytime and out-of-hours numbers supplied by your treating hospital. For severe breathing difficulty, collapse, a seizure or severe chest pain, call 112 or 999. Do not use the example numbers in this draft.

Try explaining the plan back: Explain your starting dose and usual schedule. Explain what to do after a missed dose or appointment. Name your next tests and your urgent contact route.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

One injection every 12 weeks

Active medicines: Leuprorelin 22.5 mg

Emetogenic risk: Minimal

No routine prophylactic antiemetic order. Agree symptom treatment if needed.

Additional phase-specific support

- A trained healthcare professional prepares and gives this specific formulation under the skin. Correct mixing and administration matter for hormone suppression. Different leuprorelin products and strengths have different instructions; they must not be substituted by matching the number of milligrams alone.
- Repeat blood count, kidney and liver tests, glucose and bone blood profile when clinically indicated. PSA and testosterone assess response. A suspected preparation or administration problem needs review and a testosterone check.
- Confirm the exact next injection date. With radiotherapy, ask when hormone treatment starts and ends and how the radiotherapy dates fit. A low PSA does not mean you should stop injections. Tell the team about worsening pain, trouble passing urine or severe headache with vision changes.
- Bone health, calcium and vitamin D according to individual needs and local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

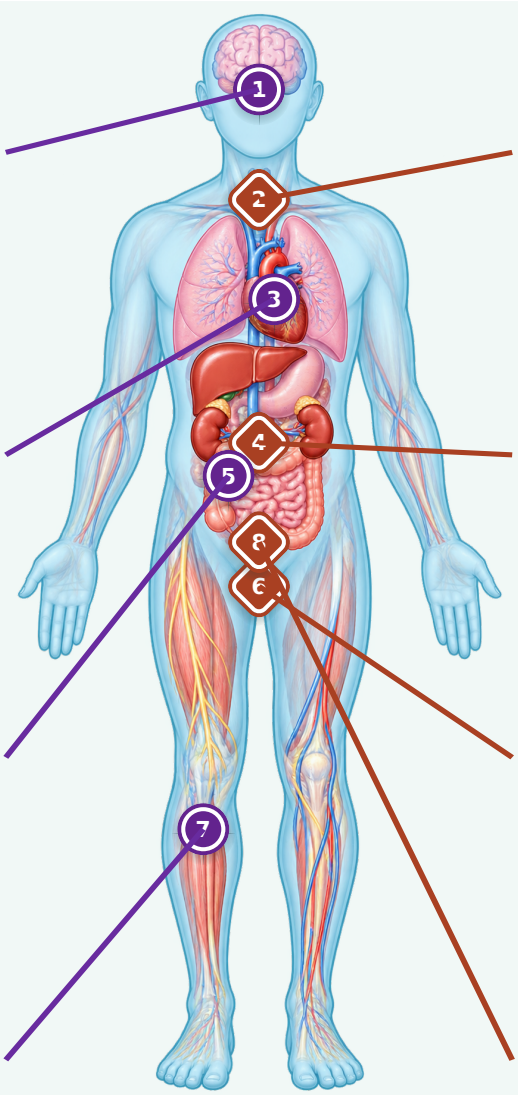
You do not need to know which medicine caused a symptom before calling.

1 Mood, sleep and energy
Leuprorelin 22.5 mg
Report severe low mood, marked fatigue or sleep problems. Ask for support early.

3 Heart and blood pressure
Leuprorelin 22.5 mg
Keep planned checks. Chest pain, fainting or severe breathlessness needs urgent assessment.

5 Injection or implant site
Leuprorelin 22.5 mg
Report worsening redness, swelling, severe pain or a persistent reaction.

7 Bones and muscles
Leuprorelin 22.5 mg
Ask about bone assessment, resistance exercise, falls prevention, calcium intake and vitamin D needs.



2 Hot flushes and sweating
Leuprorelin 22.5 mg
Hormone suppression can cause hot flushes. Ask about practical measures and treatments if troublesome.

4 Weight and blood sugar
Leuprorelin 22.5 mg
Blood glucose, weight and cardiovascular risk need review; suitable activity and diet can help.

6 Sexual health and fertility
Leuprorelin 22.5 mg
Reduced desire and erection difficulties are common. Discuss fertility and contraception; ADT is not reliable contraception.

8 Urine and early symptoms
Leuprorelin 22.5 mg
Report difficulty passing urine or worsening pain. New leg weakness or loss of bladder/ bowel control is an emergency.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call your team promptly for new or worsening symptoms, severe low mood or problems taking treatment. New leg weakness, loss of bladder or bowel control, severe chest pain, severe breathing difficulty, collapse or a seizure needs emergency assessment: call 112/999.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00479 v3: official regimen**
   https://healthservice.hse.ie/documents/6494/479_v3_Leuprorelin_22.5mg.pdf
2. **Eligard: product information**
   https://assets.hpra.ie/products/Human/25689/Licence_PA0812-005-002_18102024115227.pdf
3. **HSE: cancer treatment side effects**
   https://www.hse.ie/conditions/cancer/if-you-have-side-effects/
4. **NCCP supportive-care antiemetics for NCIS**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **NCI: hormone therapy for prostate cancer**
   https://www.cancer.gov/types/prostate/prostate-hormone-therapy-fact-sheet